

LPL - PRODUCTION TEST COLLECTION
CENTRE
SECTOR - 18, BLOCK-E ROHINI
DELHI 110085

Name	: DUMMY	Collected	: 29/5/2021 1:37:00AM
Lab No.	: DUMMYB142	Received	: 29/5/2021 4:26:47PM
Age: 25 Years	Gender: Male	Reported	: 1/6/2021 1:27:29PM
A/c Status : P	Ref By : DR. DUMMY DUMMY	Report Status	: Final

Test Name	Results	Units	Bio. Ref. Interval
MICROALBUMIN/ALBUMIN : CREATININE RATIO (ACR), URINE (Immunoturbidimetry, Compensated Jaffe's reaction, IDMS traceable)			
Microalbumin	32.00	mg/L	
Creatinine	23.00	mg/dL	24.00 - 392.00
Microalbumin:Creatinine Ratio	139.13	mg/g Creat	<30.00
ACR Category	A2 (Moderately increased)		

Note

1. Due to high biological variability and non-renal influences, ACR>30 mg/g creatinine in a random urine sample should be confirmed with a subsequent early morning urine sample or 24 hours urine sample.
2. The diagnosis of albuminuria requires the demonstration of increased albumin loss (either increased albumin creatinine ratio or albumin loss in 24 hrs urine sample) in at least two out of three urine specimens collected in the absence of infection or acute metabolic crisis.
3. The term Microalbuminuria is misleading as it implies a small version of albumin molecule rather than an excretion rate of albumin greater than normal but less than that detected by routine method. It is recommended to use term Albuminuria or Albumin Creatinine ratio (ACR) instead of Microalbuminuria.

Comments

Albumin creatinine ratio (ACR) in urine is a sensitive and specific measure of kidney damage. Urinalysis for albuminuria has been accepted as a useful way of identifying patients at risk of progressive Chronic Kidney Disease (CKD). Increased urinary albumin excretion is highly predictive of Diabetic Nephropathy, End-stage renal disease, Cardiovascular mortality and total mortality in patients with Diabetes Mellitus.

Non-Renal causes of increased ACR:

Menstrual contamination, Uncontrolled Hypertension, Urinary Tract Infection, Heart failure, Strenuous exercise and other transitory illnesses.

Usage

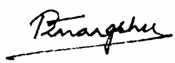
- Marker for classification of CKD & its progression



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Test Name	Results	Units	Bio. Ref. Interval
• To screen Diabetic Nephropathy			



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-----End of report -----

IMPORTANT INSTRUCTIONS

*Test results released pertain to the specimen submitted.*All test results are dependent on the quality of the sample received by the Laboratory.
 *Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician.*Sample repeats are accepted on request of Referring Physician within 7 days post reporting.*Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.*Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.*Test results may show interlaboratory variations.*The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).*Test results are not valid for medico legal purposes. *Contact customer care Tel No. +91-11-39885050 for all queries related to test results.
 (#) Sample drawn from outside source.

